



Armada Kalamunda Group

Sexual Assault Guideline

1. Purpose

The purpose of this guideline is to take a consistent approach to managing the clinical needs of people who have experienced sexual assault presenting to Armada Kalamunda Group (AKG).

Patients may present directly to the Emergency Department (ED) following an alleged sexual assault or be referred to ED by the Sexual Assault Resource Centre (SARC). Patients will only be referred to ED if SARC suspects an injury, acute psychiatric illness, or intoxication which could not be safely managed at SARC. In these circumstances the patient should be assessed in ED prior to a referral being made to the SARC duty doctor.

2. Applicability

This guideline applies to all staff working within the Emergency Department at AKG.

This guideline applies to patients over the age of 13 years.

3. Terminology

Sexual Consent	Consent can only be given when someone is of legal age (16 years for all genders in WA). It must be free and voluntary, without fear, coercion, intimidation or anything else that inhibits free agreement Consent cannot be given if the person: • Feels threatened, forced or afraid • Is restrained against their wishes • Is asleep or unconscious • Is affected by alcohol, medication or another drug to the point that they cannot consent • Is mistaken about the nature of the act or the identity of the person they are having sex with
Sexual Assault	Any sexual behaviour or act which is threatening, violent, forced, coercive, or exploitive and to which the person has not given consent or was not able to give consent
Early Evidence Kit (EEK)	A pre-prepared kit supplying all the equipment and instructions required to collect evidence from the patient. The kit standardises the specimens collected as well as the techniques utilised during the collection – Quick reference guide to specimen collection

Trauma Informed	Services that are alert to the possibility of the existence of trauma in the lives of all consumers/patients irrespective of whether it is known to exist in individual cases. The key principles of trauma informed care include safety, trustworthiness, choice, collaboration and empowerment.
Sexual Assault Resource Centre (SARC)	A 24-hour emergency service based on the grounds of King Edward Memorial Hospital (KEMH) in an outpatient clinic environment. Provides medical care, forensic examination and counselling support to people over the age of 13 years who have been sexually assaulted within the previous 14 days
Non-Fatal Strangulation	When a person has survived having pressure applied over the neck by any means. May also be called choking. Can occur once or many times by different means - SARC Fact Sheet

4. Guideline

4.1 Referral Pathway for sexual assault

AGE (YEARS)	REFER TO
<13	PCH Child Protection Unit (CPU)
13-15	CPU or SARC
≥ 16	SARC

4.2 SARC Role

- Accepts referrals for patients assessed as medically appropriate
- Refers patient to ED if an injury, acute psychiatric illness, and/or intoxication is suspected and cannot be safely managed at SARC
- SARC Doctors and Counsellors are available for advice at any time
- SARC Doctor and Counsellor may attend the hospital with the patient if they require continuing ED or hospital treatment
- SARC Counsellor available to explain the benefits of the SARC service directly to patient if required
- Will see people regardless of whether they intend to report to the WA Police

SARC can be [contacted directly](#) for advice by Nursing, Midwifery or Medical staff

4.3 Patient Considerations

- When assessing patients, consideration will be given to the patient's gender, ethnicity, religious needs, age and developmental stage in line with [trauma informed care](#) principles
- Supporting information for patient groups with diverse needs can be found [here](#)
- Patients disclosing sexual assault in clinical areas other than ED are to be assessed against criterion in **4.4**:
 - Patients who answer **YES** to any criterion are to be referred to ED
 - Patients who answer **NO** to all criterion are to be advised of the SARC service and referred if they are agreeable

4.4 Triage in ED

- Refer to Senior ED Doctor (Registrar/Consultant) if the patient has experienced any of the following:
 - Sustained any injuries
 - Is in pain
 - Received a blow to the head
 - Has reported a loss of consciousness
 - Has any vaginal or anal bleeding
 - Was or could have been pregnant prior to the incident
 - Any alcohol and/or drug use
 - Either currently or recently suffered any mental health problems
 - Had anything placed around the neck, interfering with breathing (i.e. [non-fatal strangulation](#))
- Where the answers are **NO** to the above, contact [SARC](#) for direct patient referral

4.5 Documentation

- All clinical information should be clearly documented in detail in the patient's medical record as may be used to complete a medico-legal report
- Use of form **AKMR1.0.9 SARC Emergency Care: History & Checklist** is highly recommended

4.6 Timing of the Assault

- If the assault was more than 2 weeks ago, and medical issues have been addressed, provide the patient with [SARC counselling information and contact details](#)
- Consider providing patient with an appropriate fact sheet from the [SARC Information Resources](#)

4.7 Type of assault

- Specifically ask the patient about vaginal, oral and anal penetration, and any implements used
- Specifically ask patients presenting after family and domestic violence physical assault, about sexual assault
- Clearly document all findings in the patient's medical record

4.8 Assessment

- Assess and treat intoxication, poisoning, injuries and psychiatric emergencies as you would any other patient
- Assess injuries specific to penetration, such as vaginal or anal pain or bleeding, and abdominal pain
- Ask about non-fatal strangulation and assess for [complications](#)
- If serious injuries are suspected, consider the need for surgical or gynaecological referral. Such referrals and any internal examinations should be in consultation with the SARC Doctor so that arrangements for an appropriate forensic examination can be made (refer to [4.13 Early Forensic Specimens](#))
- Clearly document all findings in the patient's medical record

4.9 Informed consent

- SARC Doctors may only examine and collect forensic evidence from a patient who has capacity and who can give informed consent (refer to [WA Health Consent to Treatment Policy](#))
- If the patient is under significant influence of drugs and/or alcohol notify the SARC Doctor. A forensic examination may be delayed until the person is coherent. Early evidence specimens can be collected but should not be handed over until consent can be obtained
- If there is evidence of impaired mental health, a psychiatric opinion is required prior to examination
- If intellectually impaired or unconscious, and the WA Police are involved, a forensic examination can be performed via the [Criminal Investigation Act 2006](#). If the WA Police are not involved, contact the SARC Doctor to discuss the consent process
- If the patient is less than 18 years old, discuss with the SARC Doctor

4.10 Management

- Consider:
 - Possibility of current pregnancy
 - Past or current mental health history
 - History of violence or aggression by the patient
 - Patient safety (from self, others, and/or a perpetrator)
 - If a patient is less than 18 years old, mandatory reporting of child sexual abuse is legally required (refer to the [Children and Community Services Act 2004](#))
- Once medically stable/treated, refer the patient to SARC, if agreeable
- Email: SARCDrs@health.wa.gov.au or Fax: (08) 9381 5426 a copy of the medical notes, nursing notes, any medication given, and a discharge summary

4.11 Emergency contraception

- Where the patient declines referral to SARC or is not transferring directly to SARC, emergency contraception will be considered and discussed with the patient:
 - Levonorgestrel 1.5mg as a single dose. This should ideally be given within 72 hours of the sexual assault but can be given up to 5 days later
 - Ulipristal 30mg (EllaOne). Up to 5 days post sexual assault, more effective than Levonorgestrel after 72 hours

- Perform pregnancy test in ED **prior** to giving emergency contraception

4.12 Sexually transmitted infections and blood borne virus (BBV) transmission

- Baseline investigations, consider:
 - HIV, Hepatitis B and C, and Syphilis serology
 - First void urine: Chlamydia trachomatis and N. gonorrhoeae PCR
- Discuss with SARC Doctor prior to taking any genital swabs. The following is to be performed **only** if the patient is not being seen by SARC for a forensic examination:
 - External genital examination and injury, and is to be documented in the patient's medical record
 - High vaginal swab: MC&S (or blind or self-obtained low vaginal swab (SOLVS))
 - Endocervical swab: MC&S and Chlamydia trachomatis, Trichomonas and N. gonorrhoeae PCR (or blind or SOLVS)
 - Samples taken for MC+S should also have a smear performed on a labelled glass slide
 - Throat and/or anal swabs (Chlamydia/Gonorrhoea PCR), if required
- Prophylactic treatment, consider:
 - Azithromycin 1g orally stat
 - Ceftriaxone 500mg IM or IV stat, if high risk of gonorrhoea or if the patient is unlikely to attend follow-up
 - Hepatitis B vaccination
 - Hepatitis B immunoglobulin, if not known to be immune and at high risk
 - HIV anti-retroviral therapy. Discuss with Infectious Diseases/SARC Doctor
 - Tenofovir 300mg/Emtricitabine 200mg, one tablet orally daily for 28 days if considered a high-risk assault. HIV nPEP starter pack (7-day supply) is available in ED
- Referral to a Sexual Health Clinic for all suspected or proven sexually acquired infections and for follow-up of HIV nPEP

4.13 Early forensic specimens

- Early collection of forensic specimens may be performed in ED to allow the patient to eat, drink and void, without loss of evidence
- EEK Forensic packs must be used and are available in ED. Each pack contains [full instructions](#)
- Items of clothing that may be forensically useful should be placed separately in paper bags
- Seal each sample in a labelled tamper proof bag to maintain chain of evidence
- Contact SARC Doctor for advice if required

5. Resources

- [SARC Information Resources](#)
- Non-Fatal Strangulation:
 - [Clinical Forensic Assessment and Management of Non-Fatal Strangulation](#)
 - [Non-Fatal Strangulation in Sexual Assault](#)

6. Legislative context

The following documents are required to give effect to this policy:

- [Criminal Investigation Act 2006](#)
- [Children and Community Services Act 2004](#)
- Health Department of WA:
 - [Consent to Treatment Policy](#)
 - [Coordinated Medical and Forensic and Counselling Response to Patients who Experience a Recent Sexual Assault and Present to an Emergency Department](#)
 - [Mandatory Reporting of Child Sexual Abuse Training Policy](#)

7. Supporting information

The following documents support the implementation of this policy:

- [Medication Management Procedure AKG-PRO-0255-18](#)
- [EMHS Chaperone Policy](#)
- [PCH Sexual Assault Protocol](#)
- [FSH Forensic Evidence Collection Policy](#)

8. Relevant standards

National Safety and Quality Health Service (NSQHS) Standards (second edition):

- Standard 2 – Action 2.05 Healthcare right and informed consent
- Standard 5 – Action 5.10 Screening of risk
- Standard 5 – Action 5.11 Clinical assessment

9. References

[FSFHG Sexual Assault Management FSH-ED-GUI-0005](#)

10. Document owner

[Document owner position title]

Enquiries relating to this guideline may be directed to:

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11. Review

This guideline will be reviewed and evaluated as required to ensure relevance and currency. At a minimum it will be reviewed within one (1) year after first issue and at least every three (3) years thereafter.

Version	Effective from	Effective to	Amendment(s)
3.0	25/08/2022	25/08/2025	Minor Amendments

12. Authorisation

This guideline has been authorised and issued by the [enter position title of the authoriser].

Authorised by	Xin Chua, A/Director Clinical Service
Authorisation date	25/08/2022
Published date	25/08/2022

Appendices

EEK Specimen Collection

	Government of Western Australia North Metropolitan Health Service Women and Newborn Health Service	
Early Evidence Kit Specimens		
PHONE	PHONE – <u>SARC</u> for advice: 6458 1828 (or KEMH switchboard 6458 2222)	
CONSENT	CONSENT : obtain prior to collection. Verbal consent/assent may be obtained first but formal written consent is required before handover of specimens. Original consent goes in ED notes. Give a copy of consent to patient.	
COLLECT	COLLECT– Early Evidence Kits Specimens Use accredited SARC EEK Kits	
LABEL	LABEL all specimens: Label with patient's name, DOB, date, time, site of collection and Nurse's/Doctor's name. Do not use hospital patient labels.	
SPECIMEN LIST	SPECIMEN LIST: A photocopy of the list to be put in tamper-evident bag prior to sealing. Tamper-evident bag must be <u>sealed</u> and details filled in before handover of specimens.	
HANDOVER	HANDOVER: Handover of tamper-evident bag either to Police or to patient. If patient, advise them to place the tamper-evident bag in the fridge.	
	WEE In yellow top	
		
		WIPE Wipe the area with sterile gauze. Place gauze in separate yellow top pot
SARC 2019 Early Evidence Kit Specimens ANY QUESTIONS CALL SARC		

Specimen collection does not need to be observed

Sample reason for collecting	Equipment	Procedure
Oral rinse: Mouth penetration by penis, (even if no ejaculation), and/or finger Biting the assailant	1 pair of gloves 10 mls sterile water 1 yellow topped collection pot labelled: • Oral rinse	Patient wears gloves Opens water Water in mouth, gargle & swirls around for 10 seconds Spits into pot Closes oral rinse pot
"Wee + wipe" (female) Vagina penetration actual or attempted by penis (even if no ejaculation) and/or finger  Urine for toxicology: Suspected drug-facilitated sexual assault or severe intoxication	1 pair of gloves 2 yellow topped collection pots labelled: • Urine (first part) • Vulval wipe Sterile gauze Bright yellow sticker: PLEASE FORWARD TO C.C. WA 1 Fluoro-oxalate blood tube (grey top)	Patient takes items to toilet and puts on gloves Passes 1 st part urine into urine pot Closes urine pot Opens gauze, gently wipes vulva and put gauze into 2 nd pot and closes 1 st part urine as above Nurse/Doctor to stick bright yellow sticker on urine sample Nurse/Doctor to fill the grey blood tube with <u>urine</u> and write collection time and patient's weight on label.
"Wee +wipe" (male) Penis in contact with mouth or body cavity Urine for toxicology: Suspected drug-facilitated sexual assault or severe intoxication	1 pair of gloves 2 yellow topped collection pots labelled: • Urine (first part) • Penile wipe Sterile gauze 10mls sterile water AS ABOVE	Patient takes items to toilet and puts on gloves Passes 1 st part urine into urine pot Closes urine pot Opens gauze, moistens gauze with water, wipes the shaft and tip of penis Places gauze in pot. Closes penile wipe pot AS ABOVE
Peri-anal wipe Anus penetration actual or attempted penetration of anus by penis, (even if no ejaculation) and/or finger.	1 pair of gloves 1 yellow topped collection pot labelled: • Peri-anal wipe Sterile gauze 10 mls sterile water	Patient takes items to toilet and puts on gloves Moistens gauze with water and wipes around the outside of anus. Places gauze in pot Closes peri-anal wipe pot
Blood Following suspected drug facilitated sexual assault	1 x 6ml fluoro-oxalate (grey top) blood tube Sterile water	Clean venepuncture site with water. Collect blood. Label with patient's name, weight and the exact time of collection
Wet and dry skin swabs Any Skin Patient bitten, licked or ejaculated on by assailant. Around mouth skin: Penile-oral penetration	1 pair of gloves 10 mls sterile water 2 plain swabs labelled: • Skin swab wet • Skin swab dry	Healthcare worker wears gloves. Opens water & swab labelled "wet". Puts a few drops of water <u>on</u> swab and gently rolls "wet" swab over <u>area of</u> skin to be sampled and puts swab back into sheath. Next healthcare worker opens swab labelled "dry" and gently rolls over SAME area and puts swab back into sheath. Document on swab labels area of skin that was sampled: eg left side of neck
Clothing If patient required to change clothes for any reason. (If nurse/doctor collecting—change gloves between each item)	Large paper bags Collect only what is necessary.	Patient to place each item in a separate paper bag Nurse/Doctor to label each bag and seal. Place red evidence tape across the seal.